DELFINA

**June 12, 2025**

Centers for Medicare & Medicaid Services
Department of Health and Human Services
Attention: CMS-0042-NC
Mail Stop C4-26-05
7500 Security Boulevard
Baltimore, MD 21244-1850

Submitted Electronically via Regulations.gov.

**RE: Public Comment on CMS-0042-NC: Request for Information–Health Technology Ecosystem**

Dear Administrator Oz and CMS RFI Review Team,

Delfina Care Inc. (Delfina) welcomes the opportunity to respond to the Centers for Medicare and Medicaid Services (CMS) Request for Information regarding the Health Technology Ecosystem (CMS-0041-NC). We are encouraged by your leadership in shaping a digital infrastructure that enables more effective and interoperable healthcare.

## Executive Summary

Delfina's mission is to transform pregnancy care by providing access to individualized insights and integrated systems that improve health outcomes. Delfina partners with managed Medicaid plans, provider organizations, and community-based healthcare practices to deliver our AI-powered maternal health technology platform. Our tech-enabled, closed-loop system of care helps healthcare providers identify patients who would benefit from early intervention, and then deploy evidence-based interventions and non-clinical support to the patient between clinical visits to help mitigate the risk for complications such as preeclampsia and preterm birth; leading drivers of maternal mortality and neonatal ICU (NICU) admissions.

We fully support CMS's goals to promote interoperable, patient-centered technologies and urge CMS to explicitly prioritize maternal health use cases across its digital health modernization strategy. In the following comments, we offer recommendations based on our experience developing and deploying scalable digital tools that integrate into clinical workflows and support real-world impact in Medicaid and value-based care.

## Delfina Responses

While not directly responding to any specific RFI question, we want to acknowledge that without a national reimbursement framework, any progress made in embracing the use of technology to serve patients remains incomplete. A unified billing policy (e.g., a dedicated CPT code for AI-enabled maternal health platforms) is crucial for fully realizing the potential of technology and supporting meaningful engagement from both providers and developers. Please find below our responses to specific questions posed.

DELFINA

## Section B. Patients & Caregivers

### 1.1 Patient Needs (PC-1 – PC-7)

Digital health tools like Delfina Care are most effective when they simplify complex care journeys, such as pregnancy, by coordinating care, surfacing timely guidance, and enabling trusted communication and connection. Currently, health information is fragmented across community obstetric (OB) practice portals, hospital and health system portals, Medicaid plan systems, Women, Infants, and Children Nutrition Program (WIC), and other community service providers' records, as well as paper charts. This fragmentation limits patients' ability to understand their care, make informed decisions, or manage chronic or urgent conditions. Consolidated access, especially during care transitions or emergencies, would enable better outcomes and reduce administrative burden for caregivers managing multiple dependents. While some care navigation apps exist for commercial-plan beneficiaries, Medicare, Medicaid, and dually eligible populations lack access to scalable, effective tools.

**Critical missing features in today's tools include passive data collection, ambient sentiment analysis, intelligent triage, integrated care coordination, and real-time insight into covered benefits.** Tools that can ingest Medicaid plan rules, provider availability, and local resources to offer proactive navigation are urgently needed but face structural barriers, including data interoperability challenges, misaligned incentives, and a highly complex regulatory framework.

CMS is uniquely positioned to enable cross-entity care coordination workflows by supporting interoperable platforms that bridge Medicare, Medicaid, and social services. To increase the adoption of digital tools, CMS could establish a trusted clearinghouse of vetted products, evaluate tools based on accessibility, usability, and their impact on outcomes, and promote adoption through reimbursement incentives, beneficiary auto-enrollment, and simplified onboarding processes.

**Improving access to CMS and provider-generated data requires investments in open Application Programming Interfaces (API), standardization of Social Determinants/Drivers of Health (SDOH) data, and enhanced technology infrastructure.** Accessibility features must go beyond interface design and should include multilingual content, low-data modes, human support options, and endorsement by trusted providers to build confidence among patients and caregivers.

**Finally, CMS should invest in real-world evidence generation by supporting embedded research partnerships with digital health vendors, encouraging voluntary reporting via application programming interfaces (APIs), integrating patient-reported outcomes into care workflows, and incorporating digital health usage metrics into the Consumer Assessment of Healthcare Providers and Systems (CAHPS) and Healthcare Effectiveness Data and Information Set (HEDIS) frameworks to evaluate effectiveness at scale**.

### 1.2 Data Access & Integration (PC-8 – PC-12d)

Patients who engage with Delfina Care typically have access to limited structured data through the patient engagement app, including remote monitoring readings, appointment reminders, and patient-input health data such as pregnancy symptoms and mood logs. Caregivers may view the specific data the patient has authorized them to see. For example, mood logs but not weight trends.

High-value data that remains difficult for patients, caregivers, and digital health vendors to access includes clinical notes (particularly OB hospitalist documentation from delivery events), historical pregnancy

complications (often buried in scanned documents), encounter-level information from community-based services (e.g., WIC, doulas, behavioral health), real-time provider availability, and social determinants/drivers of health. These limitations hinder our ability to build whole-person, culturally responsive prenatal and postpartum care plans.

Beyond traditional clinical data, access to Medicaid claims data, eligibility information, covered maternity benefits (including doula services, remote patient monitoring devices, and behavioral health services), and prior authorization status is crucial for ensuring proactive care navigation, particularly for underserved populations. To address the persistent gaps in maternal health data interoperability and patient access, we recommend that CMS:

1. **Establish a reimbursement framework to support comprehensive data collection and integration for maternal health.** Without a reimbursement framework that financially incentivizes providers to establish comprehensive data on maternal health, guidance on interoperability will have a limited impact. Such a reimbursement framework could encourage providers to collect key data elements, including estimated due date (EDD), gestational age, gravidity/parity, blood pressure, blood glucose, weight, symptoms, fetal growth metrics, perinatal mental health assessments, and peripartum clinical complication diagnoses.

2. **Integrate Medicaid benefits and eligibility data.**
   Ensure digital solution vendors can access Medicaid eligibility, plan enrollment, and covered maternity benefits, such as doula care, transportation, and behavioral health, in a standardized, FHIR-accessible format.

3. **Prioritize maternal health in the United States Core Data for Interoperability Expansion (USCDI+).**
   Include structured, pregnancy-specific clinical data elements, such as estimated due date (EDD), gestational age, gravidity/parity, blood pressure trends, fetal growth metrics, and perinatal mental health assessments, as required fields in future updates to the USCDI+.

1.3 Information Blocking & Digital Identity (PC-13 – PC-14f)

Timely access to health information is essential during pregnancy and postpartum; however, patients and providers frequently encounter information blocking, such as inaccessible lab results, discharge notes, or referrals, due to the fragmentation of information across obstetric practices, hospitals, behavioral health providers, and Medicaid plans. For pregnant individuals, these delays can be life-threatening. CMS can facilitate reporting by embedding mobile-friendly, plain-language education on information blocking into trusted care settings (e.g., obstetrician offices) and making a reporting tool that is readily integrated into third-party digital platforms. However, reporting must be tied to visible enforcement and outcomes. Without accountability, increased complaints could further erode trust. But when used effectively, complaints can help CMS surface systemic barriers, particularly during critical maternal care transitions like hospital discharge or management of hypertensive disorders of pregnancy.

## Section C Providers

2.1 Digital Health Apps (PR-1 – PR-4)

To accelerate the adoption of digital health tools among maternal health providers, particularly in rural and underserved communities, CMS should align solutions with value-based care incentives, reduce documentation burdens, and support integration into existing clinical workflows. Tools that assist with risk identification (e.g., gestational diabetes, preeclampsia, perinatal mood disorders, and substance use disorder relapse), cross-setting care coordination, and patient engagement are especially valuable in maternal care by facilitating prevention of these complications.

However, adoption faces persistent barriers, including Electronic Health Record (EHR) data fragmentation, limited reimbursement, staffing constraints, and a lack of trust in digital tools that are not tailored to obstetric care. Providers need clear, patient-facing communication about how virtual tools support pregnancy, what data is used, and who to contact for help. Clinically, providers are responsible for ensuring tools are evidence-based, culturally appropriate, and supplement, not replace, trusted care relationships.

Beyond clinical engagement, digital solutions can alleviate administrative strain; however, current workflows remain burdened by outdated quality reporting, non-standard documentation templates, and inadequate support for maternity-specific billing models. CMS can mitigate this by funding maternal health-specific tools (e.g., postpartum checklists, SDOH screeners), promoting FHIR-based reporting frameworks, and tying incentives to clinical outcomes.

Full access to both structured and unstructured EHR data, such as scanned referrals, faxed postpartum instructions, or free-text OB notes, is essential for safe, coordinated care. Inaccessibility of these records leads to delays and errors, particularly during high-risk pregnancies or postpartum transitions. While technical and privacy challenges exist, full access to this data is a high-priority need to achieve interoperability. The cost of inaction is measured in the increasing burden of preventable complications..

Finally, CMS should support standards that allow approved third-party vendors to access provider directories, intake forms, and scheduling tools with patient consent. Patients today often have to repeat the same forms across visits, providers, and systems, which is a burden for everyone, not just beneficiaries of Medicaid and Medicare.

2.2 Data Exchange (PR-5 – PR-8c)

Delfina Care supports maternal health providers and payers through data-driven care coordination, population stratification, and digital patient engagement. Our interoperability approach leverages FHIR APIs, including via

intermediaries (e.g., Metriport), to access structured clinical data critical to delivering personalized and proactive maternal care.

We currently use the Patient Access API to ingest labs, vitals, and problem lists through individual queries. The Standardized API for Patient and Population Services is in progress for gestational cohort-level data. Bulk FHIR capabilities, including Group ID-based filtering, are planned to support outreach and quality reporting for risk-stratified prenatal populations.

Key APIs that remain unsupported but are highly needed in maternal health include:

- Payer-to-Payer API, to maintain continuity of care during Medicaid churn;
- Prior Authorization API, to expedite timely access to covered services (e.g., ultrasounds, doulas);
- Provider Directory API, which would benefit from a centralized, maternal care-specific directory;
- Secure Modular Application Resource Transformation on FHIR (SMART on FHIR) and Health Level Seven (HL-7) Clinical Decision Support (CDS) Hooks, which hold promise for decision support (e.g., screening for hypertensive disorders of pregnancy (HDP)) but face adoption and EHR integration barriers.

Currently, TEFCA has a limited impact on maternal health workflows due to gaps in participation among OB clinics, Medicaid managed care organizations (Medicaid MCOs), and community support services (e.g., WIC and behavioral health). CMS should incentivize participation from these stakeholders, require alignment with EHR vendors, and prioritize structured maternal care bundles in Qualified Health Information Network (QHIN) exchanges. In the near term, targeted FHIR APIs, community HIEs, and payer-provider partnerships offer more actionable routes to interoperability.

To reduce provider burden while advancing interoperability, CMS should:

- Standardize maternal data fields (e.g., gestational age, parity, SDOH).
- Allow maternity providers to meet data-sharing requirements through approved digital care platforms.
- Align data interoperability with existing quality measures (e.g., postpartum visit rates, NCF 1517).
- Support risk-based reimbursement models tied to data completeness and referral closure.

Bulk FHIR exports to CMS could reduce quality reporting burden, but readiness varies across maternity care settings, and many gestational fields remain unstandardized. Dual-purposing digital care management data (e.g., prenatal visit tracking, blood pressure readings) for both clinical care and quality metrics would improve efficiency. CMS should also require maternal health registries to enable real-time clinical access, adopt harmonized data elements, and provide FHIR-based reporting to streamline compliance.

### 2.4 Information Blocking (PR-12 – PR-14)

Timely and complete access to electronic health information is crucial for prenatal care providers coordinating care across OB clinics and FQHCs, hospitals, behavioral health services, and Medicaid MCOs. However, information blocking continues to create critical gaps that delay diagnosis, disrupt continuity of care, and undermine outcomes.

To address this, Delfina recommends that CMS revise information blocking exceptions to reflect the realities of prenatal and maternity care. Current provisions, such as “preventing harm” or “infeasibility,” are too broadly interpreted and often used to withhold essential data, including prenatal labs and postpartum discharge summaries. CMS should clarify these exceptions, increase transparency when they’re invoked, and prioritize the structured exchange of perinatal-specific data (e.g., gestational age, pregnancy history, SDOH).

For provider categories without existing disincentives to information blocking, CMS should consider proportionate penalties tied to value-based payment models, such as reducing incentives or exclusion from

funding for digital maternal care initiatives. Technical support should accompany any enforcement for small or rural practices to facilitate compliance.

To increase provider reporting of information blocking, CMS should offer EHR-integrated complaint pathways, protect whistleblowers, and publish clear outcomes from enforcement actions. When tied to visible accountability, reporting can expose systemic barriers and advance proactive data exchange in maternity care. However, simply reporting risks, without subsequent action or follow-through from CMS, can lead to increased disengagement from providers who are already facing significant workload demands.

**Section D Payers** (PA-1 – PA-7)

To improve maternal health outcomes through interoperability, several key barriers must be addressed. TEFCA's current structure lacks support for pregnancy-specific data (e.g., gestational age, prenatal milestones) and is not well integrated with Medicaid systems or community-based maternal health programs. Without these enhancements, payers have limited incentive to participate.

CMS can accelerate API adoption by tying implementation to Medicaid MCO contracts and maternal health value-based incentives, particularly for exposing maternity benefits, authorizations, and coordination tools.

A nationwide provider directory with FHIR endpoints and digital credentials would enhance maternal referral accuracy, automate benefit and claims workflows, and facilitate effective care coordination for high-risk pregnancies.

Payers are increasingly interested in using EHR-based bulk data extraction to streamline maternal quality reporting. CMS can promote alignment across quality frameworks and encourage the development of dual-use infrastructure that supports both retrospective metrics and real-time provider insights.

Finally, CMS should encourage payers to report information blocking incidents related to maternal data access, linking this to maternal health goals while ensuring fair and constructive resolution processes.

**Section E Technology Vendors, Data Providers, and Networks**

As a maternal health technology platform serving Medicaid members, Delfina Care strongly supports the expansion of standards-based interoperability, API accessibility, and accountability in health IT. These reforms are crucial for delivering timely, coordinated care in high-risk pregnancies and for supporting improved maternal outcomes.

4.1 Ecosystem (TD-1 – TD-2d)

We recommend that CMS expand support for pregnancy-specific use cases within existing APIs and payment models, aligned with Capability Maturity Model Integration (CMMI) initiatives. This includes funding pilot programs and developer sandboxes that support longitudinal perinatal records, high-value additions to CMS APIs include SDOH, Medicaid eligibility and enrollment data, maternity benefit design, and prior authorization status. These datasets, when paired with Blue Button 2.0, can help close care coordination gaps. Current obstacles include siloed systems, a lack of standard formats, and limited access for delegated caregivers. CMS should prioritize the development and adoption of APIs, such as Data at the Point of Care, FHIR for Prior Authorization, and a structured USCDI exchange tailored to maternal health workflows.

4.3 Technical Standards & Certification (TD-4 – TD-11)

To improve interoperability in maternal health, CMS should require standards-based, open APIs in Medicaid managed care IT contracts and value-based care models to discourage the use of proprietary interfaces. A

national, free-to-access FHIR endpoint directory, ideally, maintained by ONC or a public-private entity, would streamline provider discovery and care transitions. While TEFCA's value lies in trust framework governance, redundant protocols (e.g., IHE XCA) should be consolidated. USSCDI has advanced standardization but lacks critical perinatal elements, such as tracking pregnancy episodes and determining gestational age. These should be added in phased updates, with optional support for language model-based formats validated for accuracy. Certification criteria should emphasize end-to-end data availability, structured maternal data, and patient-directed exchange of information. CMS should prioritize API-enabled capabilities while mitigating provider and EHR vendor burden through incentives for structured reports and bulk maternal quality data exchange. Developers must be required to enable full EHR access, including scanned and unstructured data, without additional contracts. CMS should also standardize Electronic Health Information (EHI) export formats (e.g., FHIR ND-JSON), support event-based triggers, and fund real-world implementation through Medicaid IT investments.

4.4 Data Exchange (TD-12 – TD-17)

To strengthen data exchange in maternal health, CMS should endorse non-CMS networks that meet clear standards, such as FHIR compatibility, support for USCDI plus maternal-specific data, and inclusion of safety-net providers. Full EHI access is critical to enable pregnancy episode tracking, behavioral health, and substance use disorder integration, and coordination with social care. While USCDI offers structure, it lacks the depth needed for maternal care; full EHI must be paired with processing tools and incentives to be actionable. Defina accesses data via FHIR (g) (10) endpoints, payer APIs, and HIEs, using NPI/Org ID search and private agreements, while monitoring TEFCA's evolving role. Expanding bulk FHIR use would improve maternal risk stratification and quality measurement, but requires standardized extraction protocols to prevent inconsistency and system strain. Point-to-point models hinder scalability. CMS should promote API unification and reciprocity to improve onboarding and access. Finally, CMS and ONC should co-invest in open maternal health data infrastructure and incentivize payer participation, using maternal outcomes metrics (e.g., postpartum visit attendance, detection of hypertensive disorders of pregnancy) to drive demand and real-world adoption.

4.5 Compliance (TD-18 – TD-19)

Persistent information blocking, such as hospitals withholding obstetric outcomes data and EHR vendors charging fees for access to structured prenatal data, continues to undermine maternal care coordination, especially in safety-net settings. CMS should enforce more substantial disincentives for actors who impede data sharing critical to pregnancy and postpartum care. Additionally, current price transparency tools often exclude maternity bundles and ancillary services, such as doula care. CMS should mandate standardized episode-based maternity pricing and enable real-time API access for providers, payers, and vendors. Targeted innovation grants and Medicaid MCO IT requirements would further accelerate the adoption and usability of these solutions.

## Section F Value-Based Care Organizations

5.1 Digital Health Adoption (VB-1 – VB-4)

To support the broader adoption of digital health tools in value-based care models, such as Accountable Care Organizations (ACOs) and the Medicare Shared Savings Program, CMS should align incentives with the use of certified digital care navigation, remote monitoring, and care management platforms that improve access and outcomes, particularly in maternal health. This includes offering quality bonus points, shared savings multipliers, or startup grants for digital tools that demonstrate impact on engagement, outcomes, and cost.

Current barriers to adoption include a lack of EHR interoperability and limited reimbursement for non-clinical providers, such as doulas, behavioral health professionals, and community-based organizations, which are crucial to improving maternal care.

CMS should integrate digital innovation themes, such as AI-driven risk stratification, care coordination, and population health analytics, into the core Alternative Payment Model (APM) requirements. These technologies must be integrated into APM performance frameworks, with expectations for usability, patient engagement (e.g., via mobile access), and clinical decision support, particularly for high-risk and underserved populations.

Essential IT capabilities for maternal health within value-based arrangements include:

- Gestational-age-adjusted care planning and pregnancy episode tracking
- Patient event notifications (e.g., hospital admission, delivery, ED visit)
- SDOH-informed risk scoring and remote monitoring device interoperability (e.g., blood pressure cuffs, glucometers)
- Attribution and patient-matching tools
- Integration of claims, authorization, and benefit design data
- Real-time quality performance dashboards

Data types critical for success include structured data, prior pregnancies, behavioral health status, SDOH assessments, benefit coverage (e.g., doula services), and claims data for utilization and cost trend analysis.

### 5.2 Compliance & Certification (VB-5 – VB-10)

Current ONC Health IT Certification Program criteria offer a useful baseline but fall short in supporting value-based maternal care. They lack support for episode-based care planning, pregnancy-specific data elements, and integration with community providers and digital care navigation platforms.

CMS should expand certification criteria to include structured maternal and perinatal data elements (e.g., gestational age, risk flags), care coordination functionality across both clinical and non-clinical teams, and API-based access to benefit and SDOH data. Certification should also prioritize bulk data export and real-time event-based reporting for maternity quality metrics.

Technology requirements for APMs should strike a balance between flexibility and clarity, providing use-case-based guidance that supports both flexibility and clarity. For example, maternity care providers in Advanced APMs should not be required to implement the same features as oncology practices. However, they should be required to support gestational risk tracking and visibility of postpartum follow-up.

Other Health and Human Services (HHS) policies, such as Conditions of Participation, can reinforce Certified Electronic Health Record Technology (CEHRT) by mandating hospital notifications to APM-participating OBs, midwives, and care coordinators. Currently, event notifications often fail to reach community-based maternity providers due to EHR silos and a lack of contact registry infrastructure.

Technology requirements for APMs should go beyond those for fee-for-service providers, emphasizing quality performance tracking, care coordination capabilities, and broader data sharing. For smaller or rural APMs, CMS should support flexibilities like modular CEHRT adoption, shared IT infrastructure, and funding support to meet requirements without compromising data quality or patient safety.

### 5.3 Technical Standards (VB-11 – VB-15)

Key interoperability challenges in implementing value-based maternity care include difficulties accessing longitudinal pregnancy records across different systems, delays in receiving hospital discharge summaries, a

lack of standard formats for SDOH data, and limited integration with Medicaid-specific systems. These issues hinder coordinated care, timely interventions, and accurate quality reporting.

To balance innovation with flexibility, CMS should advance standardization by requiring FHIR-based APIs for all certified systems, while allowing clinical content customization for different APM types. For maternal health, this could include episode-specific data bundles (e.g., prenatal labs, delivery outcomes) and optional modules for community-based provider integration.

Improvements to certification criteria should include enhanced support for bulk data export, maternal episode tracking, and SDOH-aware analytics. Reducing provider burden requires that these functions work seamlessly within existing clinical workflows, without requiring duplicative documentation.

## Closing

At Delfina Care, we believe every pregnancy deserves proactive, coordinated, and data-informed support, especially for those served by Medicaid and safety-net providers. Our experience working with health plans, providers, and digital infrastructure partners has shown that when maternal health data is timely, structured, and shared, we improve both costs and outcomes.

We appreciate the leadership of CMS and ONC in advancing policies that strengthen interoperability, promote digital access, and align value-based care models with the needs of pregnant and postpartum individuals. With maternal morbidity and mortality rates continuing to rise, especially among Medicaid-covered individuals, CMS's digital health policies have the power to reverse this trend by fostering more connected, proactive, and compassionate care. We encourage CMS to continue prioritizing maternal health by considering our recommendations detailed above and by pursuing a national reimbursement framework to fully realize the potential of technology.

Thank you for the opportunity to contribute to this important conversation. We look forward to continued collaboration to build a health technology ecosystem that enables better maternal health for every community in the United States. Please reach out to Delfina CEO Senan Ebrahim, MD, PhD, at ceo@delfina.com with any questions or for additional information.

*Submitted by Delfina Care Inc.*
www.delfina.com
ceo@delfina.com